

Johnston and Johnston v. Standard Life Assurance Co.,
Royal Trust Corp. of Canada, 136738 Canada Ltd.
and C.A. Fitzsimmons & Co.

Indexed as: Johnston v. Standard Life Assurance Co.
(H.C.J.)

73 O.R. (2d) 495
[1990] O.J. No. 990
Action No. 8276/87

ONTARIO
High Court of Justice
McRae J.
June 12, 1990.

Torts -- Negligence -- Occupier's liability -- Statutory duty of care -- Resident tripping on mat in apartment building parking garage -- Mat behind entrance door not fixed to floor and buckling up on opening of door -- Owner liable for hazard -- Occupiers' Liability Act, R.S.O. 1980, c. 322.

Torts -- Negligence -- Contributory negligence -- Conduct constituting -- Resident of apartment building aware of loose mat behind entrance door of parking lot -- Resident tripping when walking straight ahead without looking for hazard 50 per cent responsible for injuries.

Damages -- Personal injuries -- Non-pecuniary loss -- Quantum -- Hip fracture resulting in severe restriction of activity -- Fracture also accelerating onset of spinal stenotic condition -- General damages \$70,000 for fracture and both replacements and \$10,000 for acceleration of stenotic condition.

Damages -- Personal injuries -- Pre-existing condition -- Hip fracture accelerating onset of spinal stenotic condition -- Severe restriction of activity, second fall due to leg failure, fracture of other hip, and pain from degenerative condition not attributable to original fall and first fracture.

The plaintiff broke her hip by falling on a loose mat placed outside the access door to the parking garage of her apartment building. She had lived in the apartment building for seven years and was well aware of the existence of the mat and that it was loose and often buckled and slipped out of position. On the occasion of her injury, she entered the parking lot through the access door without pausing or looking to check the condition and placement of the mat. She tripped on the edge of the mat, which had buckled, and fell, breaking her right hip. The hip was surgically repaired, but necrosis developed after about three years and a complete hip replacement was required. During this period, she also developed spinal stenosis, allegedly partially as a result of added stress to her spine caused by the hip injury, and will require surgery to correct this condition. She may require a second right hip replacement. She also fell a second time, due to the symptoms of the spinal stenosis, and fractured her left hip. The fracture was reduced surgically and phlebitis, a common complication, followed. After each fracture, she was significantly restricted in her activities and experienced considerable pain. She is disabled, often requires painkillers and needs assistance in performing most normal activities.

Held, judgment for plaintiff with equal apportionment of liability.

The mat constituted a hazard in the circumstances and in its location. The owner of the apartment building was in breach of its duty to take reasonable care to make the premises reasonably safe for the users. The plaintiff, however, was aware of the location and condition of the mat, had used the access door for many years, and should have taken greater care for her own safety. The plaintiff had by her own negligence contributed to the accident. In this case, the plaintiff was 50

per cent responsible and the defendant 50 per cent responsible for the plaintiff's injury.

As to damages, the plaintiff had suffered significantly from the original fracture. For pain and suffering, including the original surgery, the first hip replacement and the future second hip replacement, and for the permanent restriction on her activities, \$70,000 was awarded. In addition, \$10,000 was awarded for the accelerated onset of the spinal stenosis. The first fracture, however, was not the cause of the spinal stenosis or of the second fracture, and the effects of these problems were not attributable to the original fracture.

Waldick v. Malcolm (1989), 70 O.R. (2d) 717, 63 D.L.R. (4th) 583, 35 O.A.C. 389 (C.A.) [application for leave to appeal to S.C.C. filed January 22, 1990], consd

Statutes considered

Family Law Act, 1986, S.O. 1986, c. 4

Negligence Act, R.S.O. 1980, c. 315

Occupiers' Liability Act, R.S.O. 1980, c. 322

ACTION for damages for personal injuries and for damages for loss of care, guidance and companionship pursuant to the Family Law Act, 1986.

Peter T. Bishop, for plaintiffs.

Eric R. Williams, for defendants.

MCRAE J.:-- The plaintiff, Betty Johnston, sues for damages for injuries she received when she tripped and fell on a floor mat in the parking garage of the apartment building where she lived with her husband William Johnston. Her husband, the co-plaintiff, sues for damages for loss of care, guidance, and companionship pursuant to the Family Law Act, 1986, S.O. 1986,

c. 4.

The building was owned by Standard Life Assurance Company who acknowledges that it is responsible for any negligence or breach of duty the plaintiffs may prove.

On June 29, 1984, the plaintiff, Betty Johnston, was 54 years of age. She and her husband are the parents of three adult sons, all of whom had married, left home and were raising children of their own. The plaintiffs had lived in the building known as Bromley Square, Carling Avenue, Ottawa, for about seven years prior to the accident. During that whole period their car was parked on the lower parking level known as B1 where the mat was located. They had walked over the subject mat hundreds of times.

On the afternoon of June 24, 1984, the plaintiffs picked up a young grandson at Ottawa Uplands Airport, returned to the apartment, packed and loaded their car for a trip to their cottage at Round Lake. At about 7 p.m. they descended the elevator to the B1 level, then walked along a carpeted hallway to a door which led to that area of the garage where their car was parked. This door opens out into the garage. It has no handle on the inside, but has a pressure closure mechanism which requires some strength to push open. The female plaintiff preceded her husband to the door carrying only a purse. She pushed the door open, stepped into the garage with her right foot and immediately fell. It appears her right foot went under the mat causing her to trip. She went down with the mat between her legs.

Her hip was broken in the fall and she was unable to rise. With assistance from her son and husband she was taken to the Ottawa Civic Hospital where her hip was repaired surgically. She was hospitalized for a total of 11 days.

The fractured hip was later complicated by necrosis of the head of the femur not uncommon with this type of injury. It became necessary to install an artificial hip. This was done June 26, 1987, three years after the original fracture. Her hospital stay this time lasted 12 days until July 8, 1987.

At the beginning of April 1988 after making good progress with the artificial hip she developed additional symptoms. She was diagnosed on July 11 of that year to be suffering from spinal stenosis. On August 15, 1988, while attempting to rise from a bench-style seat in a restaurant she found that her right leg had no strength. In attempting to sit down again she missed the seat, fell to the floor and fractured her left hip. This was also surgically repaired and required an additional 11 days hospital confinement. Her condition today is not good. She is in constant pain, uses a cane and is not able to walk well at all. She plans to undergo surgery to alleviate the symptoms of spinal stenosis during the summer of 1990.

Liability

In my view this is a case for an apportionment of liability. The owners of the building bear part of the responsibility for the accident. The mat was a common type used in the winter months in Ottawa and in other Canadian cities. It was 4 feet wide by slightly over 6 feet in length, a runner type mat made with a synthetic carpet fibre on a vinyl backing. It had been cut from a larger roll and can be seen to have a one inch vinyl edging on each side. At the ends where it was cut from the roll, there is no such edging. The mat was lying loosely on the concrete floor of the parking garage. As a result of dirt and grit commonly found on such a floor it was relatively easy to move. There was heavy traffic in the area. It was frequently crossed by people with shopping carts. The building apparently permitted such carts to be kept in the lobby so that residents could use them to load and unload their cars.

On the occasion in question, it had curled in some fashion, sufficiently so that the plaintiff's foot went under it causing her to trip.

It is clear from the evidence that this carpet had a tendency to wrinkle or billow up, causing a hazard.

I was impressed with and accept the evidence of Mr. John Mills, a retired sergeant of the Royal Canadian Mounted Police,

who testified that on one occasion he tripped and nearly fell on a large billow in this mat, and that on three or four occasions he straightened the mat to prevent injury to others. Mr. Gerald Palmer, currently regional safety co-ordinator for the Regional Municipality of Ottawa-Carleton and previously a private safety consultant and an expert in the field of occupational health and safety, found the rug to be a hazard. It was loose, not secured to the floor in any way, and it could be moved relatively easily because of the presence of dirt and grit which tended to collect underneath it and had the effect of preventing the vinyl backing from bonding to the floor. If the mat was one to two inches up the wall at the hinge end of the door the rug would curl and create a hazard when the door was opened, if moved up the wall on the latch side of the door it could rise up to one and one-half inches because of the cove shape seal at the bottom of the concrete wall. This also would cause a hazard. In either event no extra force would be required to open the door.

I am satisfied and accept Mrs. Johnston's evidence that she caught her foot under the mat because of the hazard it presented. This caused her to fall and caused the fracture of her right hip.

It is at the same time apparent that Mrs. Johnston's own negligence contributed to the accident. She was very familiar with this location, having lived in the building for about seven years, the mat was in the same spot over that entire period of time, and she had crossed it many hundreds of times when going to and from the family car. In spite of this, she opened the door and stepped into the garage without paying any heed to her footing. She simply looked, in her words, straight ahead.

The Occupiers Liability Act, R.S.O. 1980, c. 322, provides that an occupier owes a duty to take such care as in all the circumstances of the case is reasonable to see that persons entering on the premises are reasonably safe while on the premises. I have decided that the defendant has not met that duty. The Act also provides that the provisions of the Negligence Act, R.S.O. 1980, c. 315, apply with respect to

causes of action to which this Act applies. The plaintiff has, by her negligence, contributed to the accident.

The Court of Appeal in *Waldick v. Malcolm* (1989), 70 O.R. (2d) 717, 63 D.L.R. (4th) 583, 35 O.A.C. 389 [application for leave to appeal to S.C.C. filed January 22, 1990], supports my finding that contributory negligence by the plaintiff should be carefully considered. The court had this to say at p. 732 O.R.:

It is quite possible that other judges might have reached a different conclusion on contributory negligence than the trial judge. Before the Supreme Court decisions referred to in the above paragraph, appellate judges frequently interfered with or reversed the findings of trial judges with respect to negligence or contributory negligence because it was believed that they were in as good a position on the basis of the written record as the trial judge to draw the necessary inferences in support of their conclusions. The books are full of examples of this kind of review by appellate courts including the decision of this court in *Lewis v. Todd* (1978), 5 C.C.L.T. 167. The judges of this court and all other appellate courts are no longer free to reverse the decisions of trial court judges with respect to negligence. They can only act where it can be shown that serious errors have been made by the trial judge.

This type of mat is very commonly used, particularly in winter, to collect snow, slush and dirt from winter boots and is usually quite safe. However, under the particular circumstances of this case, and in the location where it was found, it did create a hazard. I have decided that Mrs. Johnston is 50 per cent responsible for the accident and the defendant 50 per cent responsible.

Damages

Mrs. Johnston, on the evidence, was a normal, healthy 54-year-old, prior to the accident. Since that time, with some ups and downs, her condition can be generally stated to have gone downhill rather seriously. She is now only 60 years of age

but appears quite disabled.

I propose to deal first of all with the fractured hip and the direct consequences therefrom. During the summer of 1984 after discharge from hospital, Mrs. Johnston walked, to a limited amount, and with the use of crutches but spent most of her days in a reclining chair in the apartment taking Tylenol 3's which are heavy pain medication. She was able to put partial weight on her foot by August 1984 and was bearing her full weight by November 1984.

On November 15, 1984 the couple drove to Florida to their mobile home. Her condition gradually improved and she was able to walk on the beach and perform the exercises prescribed for her. By March 1985 she was able to walk up to mile every day if weather permitted. There was still pain in her hip but she was steadily improving. She was even able to do a bit of housework and play cards with friends. In late February 1985 she played nine holes of golf using a power cart. In total she played eight or ten games before they returned to Canada. In April 1985 she began to notice a stinging feeling at the top of her right hip. The Johnstons moved into their summer place at Round Lake and she had a moderately good summer although the stinging pain increased. On August 22, 1985, the pins were removed from her hip which corrected the "stinging" feeling. They returned to Florida in November 1985 and again she was able to walk every day. She attempted to increase her activities and was optimistic that she would have a complete recovery. Her activity level improved during this period of time. She played more golf, did more housework but was still not completely better. She was able to play about ten games of golf in Florida during the winter and she was able to take daily walks on the beach of about a mile in length, but the activity seemed to bring on pain. This continued through the summer of 1986 but was relieved by anti-inflammatory medication and she was able to do most of the cooking and light housework. By that fall her right hip was getting worse and she was advised not to play golf because deterioration was noticeable. During this whole period she was on anti-inflammatory medication and Tylenol for pain. By May of 1987 she was getting very, very intense pain in her right hip and it was apparent to her doctor that the ball

of the hip had deteriorated to the point where she needed an artificial hip implant. This was done June 26, 1987.

The plaintiff noticed immediate improvement although the pain persisted. She was released from hospital on July 8, 1987, on crutches. She was on crutches for eight weeks after the surgery and a cane about an additional two months. By November she was able to walk unassisted. She was given therapy and some exercises to strengthen her leg. In Florida she walked with a cane; by March of 1988 she was able to play golf. During the winter of 1987 to March 1988, her condition gradually improved with the replacement hip and she felt her progress was very good. She had only low to moderate pain in her hip and was as good as she had been since the accident, except for the spring of 1985. In April 1988 new symptoms developed. She felt a burning pain in her buttock, groin, and from her right hip down to the knee. She reverted to the use of a cane.

On July 11, 1988, the new problems were seen to be caused by a spinal stenosis. On August 15, 1988, after having lunch in the St. Laurent Shopping Centre, she attempted to rise from the seat where she had been sitting but realized that her right leg would not hold her. She tried to sit again, but instead, she missed the seat and fell on the floor breaking her left hip. She was taken to the Ottawa General Hospital and admitted on August 15. Dr. Loehr reduced the fracture surgically. She was in hospital until August 26. She had an additional admission to hospital September 6, 1988, where she remained until September 14, with phlebitis, a not uncommon complication of the surgery she had undergone.

Recovery was slow and painful due to the stenotic condition and the after-effects of the fracture. At present she is able to walk without crutches by using a cane. Her social and recreational activities were nil during the winter of 1988 and 1989. She was unable to work or clean; her symptoms got worse. They sold their Florida property in the spring of 1989. She is now very restricted in her activities and is assisted by her husband in all of her day to day activities.

The surgery to relieve her stenotic condition, which is

scheduled to be done this coming summer, has an excellent chance of success, although there are risks associated with the surgery.

General damages for the fractured hip and its direct consequences including pain, loss of enjoyment of life, the replacement with an artificial hip and the probability that she will require a further hip replacement I have assessed at \$70,000.

Assessing damages for her problems not directly caused by the fall are much more difficult. Her problems today are caused by a natural degenerative stenotic or narrowing condition in her spine. My task is not made easier by the conflicting opinions of two eminently qualified orthopaedic surgeons.

I find that during the late winter and early spring of 1988 the distress she increasingly felt was caused by the stenosis. This was positively diagnosed by Dr. Murnaghan, July 11, 1988 when new X-rays showed degenerative disc disease at the L5/S1 level. The fall of August 15, 1988 and the left hip fracture caused thereby was due to a weakness in the right leg which resulted from the degenerative condition and was not the direct result of the 1984 accident. The phlebitis and other sequelae to this fracture and her present unhappy condition are also not the direct result of the accident.

Dr. J. Patrick Murnaghan who has been Mrs. Johnston's orthopaedic surgeon since the accident in 1984 is of the following opinion (I quote from his report of October 12, 1989):

The back symptoms are on the basis of degenerative disc disease which may have been contributed to in a significant way by the prolonged period of restricted activity since the accident of June 1984. Limping while walking using a cane or crutches all put added stress on the lower lumbar spine and I believe the pre-existing disease has been rendered more symptomatic by the stresses related to the fracture of the right hip.

Dr. Murnaghan gave similar testimony at trial. That was and is his firm opinion.

Dr. C.C. Carruthers, an orthopaedic surgeon who is at least as well qualified as Dr. Murnaghan testified for the defence. In his report on November 14, 1989, he had this to say:

This lady has several pathological problems going on. With regards to her right hip she had a fracture which developed avascular neurosis and she then went on to a total hip replacement. This sequence of events would all be related to her first accident in 1984. However, in the beginning of 1987, unrelated to her accident, she developed symptoms compatible with spinal stenosis. This is demonstrated by irritation of the nerves in her leg. This is aggravated by activities such as walking which gives her numbness and discomfort. This is what precipitated her discomfort in the mall at the time she fell and injured her hip. In other words the spinal stenosis with the nerve root irritation was the cause of the weakness in her right leg and her subsequent fall and fracture of her left hip. These symptoms are not due to her fall in 1984. As such, in my opinion there is no relationship between the fracture of her right hip and the subsequent fracture of her left hip.

Before me at trial he testified that the C.A.T. scan revealed a narrowing at L3 to L5 as well as the problem at L5/S1 which was seen on X-rays. This confirms his opinion that the hip did not aggravate the stenosis farther up the spine. The 1984 fracture could not have caused or contributed to the L3 to L5 stenosis. Under cross-examination he acknowledged the possibility but said it was highly unlikely that the fracture had any significant effect on her spinal stenosis.

Dr. Murnaghan's opinion was that the early stenotic symptoms resulted from the narrowing at L5/S1 lower down the spine than L3 to L5 and this pathology was likely aggravated by the extra stress which resulted from the accident and that the fracture resulted in her symptoms being more severe now than they would have been if there had not been the original accident.

It is always difficult for a court to assess the degree of compensation for which a tortfeasor is liable in the case of subsequent complications which are not directly related to the original accident. It is particularly difficult where there are two conflicting medical opinions.

I have analyzed the medical evidence and conclude that the stenotic condition, the left hip fracture, the phlebitis, the pain and loss of enjoyment of life resulting from these problems, including the further surgery to correct the stenotic condition which is anticipated, are not results of the accident.

The June 1984 accident, however, did have a minimal impact in that it probably contributed to an earlier onset of symptoms in a previously asymptomatic condition.

I have decided that there should be an assessment of \$10,000 as compensation for the contribution of the accident to the subsequent complications.

Pecuniary damages

The plaintiff claims large pecuniary damages both past and future. They are wide-ranging and imaginative but frequently are not related to the accident or only marginally connected and in some cases result in a financial benefit to the plaintiffs and must therefore be reduced.

I assessed these claims as follows.

Past medical costs covered by O.H.I.P. to August 6, 1987
\$16,354.27.

Additional Florida travel expenses are assessed at \$1,500.

Rental of trailer site 84/85 is disallowed as too remote.

Florida room addition at Round Lake increased the value of the asset and was enjoyed by other members of the family as well as Betty Johnston; in all of the circumstances an

assessment of \$3,000 seems appropriate.

Cost of freezer is disallowed as being too remote.

The purchase of an additional reclining chair was only partly necessitated by the accident. I would allow half of the total amount of \$252.

I would allow half of the July/August 1984 telephone expenses, specifically \$103.

Of the transportation, parking and meal expenses of the \$4,434.30 claimed I have assessed \$2,500 to the accident.

Prescription drugs not covered by other insurance and attributable to the accident I have assessed at \$1,000.

The claim for a larger and new motor vehicle is not allowed as being too remote and in any event resulting in a benefit which ought not to be assessed against the defendant as a special pecuniary loss.

The past pecuniary total therefore is \$24,709.27 of which \$16,354.27 are O.H.I.P. disbursements.

Future pecuniary damages

With respect to future health care costs, two things are immediately apparent. They may be very substantial and they are only marginally related to the accident.

As I have previously found, the plaintiff's present pathology is due to a degenerative condition. The only connection to the accident is that the onset of symptoms probably occurred sooner than otherwise because of the stress relating to the injury.

The probable future right hip replacement is the one future pecuniary loss that is directly related to the accident.

In attempting to be fair to both parties I have assessed future pecuniary loss at \$10,000.

Summary of damages

General Damages	\$ 80,000.00	Past Pecuniary	Including
O.H.I.P.	\$ 24,709.27	Future Pecuniary	\$ 10,000.00

\$114,709.27

Therefore plaintiff Betty Johnston's judgment is for \$ 57,354.63 (O.H.I.P.'s share of this is \$8,177.13)

There is ample justification in the evidence to assess the plaintiff William Johnston's Family Law Act, 1986 damages at \$10,000.00

Judgment will issue for him in the amount of \$5,000.00.

Judgment for plaintiff.

TRRT PILT